Data request to measure treatment dropout

What it is for and what the clinic receives

With the same definition at every clinic we measure **dropout per cycle**: each retrieval opens a cycle that ends in a baby, continues (a new retrieval or donor-egg cycle) or ends in dropout, when 12 months go by with no retrieval, transfer or donor-egg cycle.

In return, the clinic receives **its own report**: "of every 100 retrievals, how many end in a baby, how many continue and how many end in dropout", when patients are lost and the quality of its data. If it has several sites, the report compares them with each other; it is never compared with another organization.

Which reports to ask for: as the system exports them

No processing is needed: no calculations, no grouping, no recoding. Export each report **complete, as it comes** (Excel or CSV, with whatever column names it has). Period: every retrieval since records are reliable (ideally 3 years or more) and everything that happened afterwards.

Retrievals	Required	Patient (the same number in every report), date and retrieval type (own, donor, egg preservation, cancelled). If available: site, physician, age or date of birth, payer, prior cycles.
Transfers	Required	Patient, date, site and the outcome (negative, loss, live birth, ongoing), not just the β-hCG; here or in its own report. If available: loss date, embryo source, transfer number.
Pregnancy outcomes	Optional	Only if the outcome is not in Transfers: patient, outcome and date (or transfer number).
Donor-egg cycles	Recommended	Patient and cycle date.
Embryology by retrieval	Recommended	Patient and retrieval date (or number); per embryo, stage, PGT-A result and fate (or the counts per retrieval).
Cryopreservation	Recommended	Patient and freeze date; if available, exit date and type. If only the current inventory is available, it still helps.
Cutoff date	Required	The date through which records are complete. It can be the export date.

The minimum for the figure to be reliable

- The **same patient number** in every report; if you changed system or numbering during the period, when.
- The **outcome** of every transfer, not just the β-hCG.
- The **type of every retrieval**: own, donor, egg preservation or cancelled (or export only the own ones and tell us).
- The **site** in every report.
- The **procedure date**, not the date it was entered in the system.

Helpful optional items: prior cycles, and the date of birth if age is missing (the app works out the age at each retrieval and does not keep the date). With embryology and cryopreservation, the report separates dropouts with and without stored embryos; without them, it states this as a limitation.

The app takes them as they come

- **One file with several reports** (an Excel workbook with several sheets, or a system report with all the activity).
- **One report in several files** (by site, or from before and after a system change): they are added together.
- **One row per cycle or per patient**: the app splits it into retrievals and transfers and asks you to confirm.
- **The medical record number as the patient number**: it is replaced with an internal code when the file is read.
- If the lab uses **another patient number**, also send the equivalence table that relates the two.

If you do not have an item

- Send the report anyway, without that column, and let us know. Do not fill it in by hand: a missing item is reported as missing; an invented one distorts the result.
- If it lives in another system (for example, the lab's), export that report too, as it comes.
- If a report cannot be exported, let us know and we will work out together how to get it out: there is no need to copy it by hand into an Excel file.

Privacy: the data never leave the computer

- Everything is computed on this computer: nothing goes to the internet.
- Names, IDs, emails and phones are discarded when the file is read; if the medical record number is the only key, it is replaced with an internal code.
- The Excel that comes out has only counts and percentages, no dates or patient codes.

How it is delivered

Open the LUUMA app on the clinic's computer, drop the reports, review what the app asks and send us the **Excel for LUUMA** it generates (counts and percentages only). The clinic keeps its report.

Glossary

Dropout per cycle
The cycle did not end in a baby and 12 months went by with no retrieval, transfer or donor-egg cycle at the clinic after its last procedure. Consultations and tests do not count as procedures. It is the report's primary measure.

Retrieval
Follicular aspiration to obtain the patient's own eggs. Each retrieval opens a treatment cycle.

Retrieval type
Own: the patient's eggs for her own treatment (the only kind that opens a cycle). Donor: a retrieval from an egg donor in the donation program. Preservation: eggs vitrified for later use. Cancelled: there was no aspiration.

Cycle
A retrieval and everything that follows it: transfers of its embryos, until success, a new retrieval or donor-egg cycle, or dropout.

Transfer
Placement of one or more embryos in the uterus, fresh or with cryopreserved embryos.

Embryology by retrieval
The lab report for each retrieval: oocytes, MII, embryos, blastocysts and, per embryo, PGT-A result and fate. One row per retrieval or one per embryo both work.

MII
Mature oocyte (metaphase II): the only kind that can be fertilized.

Blastocyst
Embryo at day 5 to 7 of development. The stage at which it is biopsied for PGT-A and at which most embryos are transferred or vitrified.

PGT-A
Preimplantation genetic testing for aneuploidy: the blastocyst is biopsied to know whether it has the normal number of chromosomes.

Euploid, aneuploid, mosaic
PGT-A results. Euploid: normal number of chromosomes. Aneuploid: abnormal number; not transferred. Mosaic: a mix of normal and abnormal cells; for now it counts as viable. No result: the biopsy gave no diagnosis; it counts as untested.

Viable embryo
A blastocyst that is not aneuploid (euploid or untested). The retrieval left none if there were no oocytes, no blastocysts, or all were aneuploid. The app works it out from the embryology report.

Cryopreservation
Embryos vitrified (frozen) in the clinic's bank. The report says when each embryo went in and when it left (thawed, discarded or transferred out); from it the app counts how many are left after each transfer.

Embryos left
The patient's cryopreserved embryos after a transfer: those frozen by that day and not thawed, discarded or transferred out. Embryos thawed for that transfer count as used. It separates dropouts with and without stored embryos.

Donor-egg cycle
Treatment with donor eggs or embryos. For this measurement, moving to a donor-egg cycle counts as continuing treatment.

Pregnancy outcome
How it ended: negative (includes biochemical and ectopic), loss, live birth, ongoing at the cutoff date, or not recorded. A positive β-hCG without an outcome counts as not recorded.

Procedure date
The day the retrieval or transfer was performed, not the day it was entered in the system.

Cutoff date
The last day through which records are complete. It tells which cycles have had 12 months of follow-up. It is not the date of the last contact with each patient.

Prior cycles
Assisted reproduction treatments the patient had before her first retrieval in the period, at this or another clinic. Optional.

Patient number or medical record number
The identifier the clinic uses, the same in every report (including the lab's and the embryo bank's). If the lab uses another number, an equivalence table relating the two works. The medical record number is replaced with an internal code when the file is read and never leaves the computer.

Site
Each clinic or center of the organization where the procedure was performed. It goes in every report.

Cells under 11
To protect privacy, no row or cell in the Excel shows counts between 1 and 10: they are left blank so nobody can be identified.